جوستاف روسي انترناشيونال - مصر (EPE) *

طلب علاج أول مرة / متردد

إسم المريض رباعياً : منصوره عادلى احمد محمد
المحافظة : القاهره **الحي / المدينة :** المطريه **العنوان :** الهرم
رقم البطاقة : 26811182700122 **رقم التليفون :** 01550042567 **تاريخ تسجيل الإستمارة :** 10-10-2026
العمر(تاريخ الميلاد) : 18-11-1968 **المهنة :** بدون عمل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي المقر بما فيه
التوقيع:(----------) التوقيع:(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : أبحاث أورام ماموجرام وسونار علي الثديين [10888] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس
وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- ا.د/ ---------- د/ ---------- د/ ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: منصوره عادلى احمد محمد
National ID: 26811182700122

GUSTAVE ROUSSY

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	بشان تقرير طبى للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : ---:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي Bilateral mammogram and breast ultrasound

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين


جمهورية مصر العربية
بطاقة تحقيق الشخصية
منصوره
عادلى احمد محمد
٩ عبدالصمد عقبى اسامة ابو عمرة فيصل
الهرم - الجيزه
٢٢٠١٠٠٢٧١١٨٢٦٨
KJ2430860


٢٠٢٤/٠١
بدون عمل
انثى
مسلمة
ارملة
البطاقة سارية حتى ٢٠٣١/٠١/١٥

Patient Name	: الأستاذه / منصوره عادلى احمد	Request No.	: 13002182
Patient No.	: 26811182700122	Request Date	: 23/03/2023
Path No.	: 914/2023	Verification Date	: 09/04/2023
Sex	: Female	Financial Category	: اورام دار السلام - نقابة الدولة
Age	: 54 Y	Department	: Out Patient
Referreing Doc :		Registered by	: Ali mostafa

Pathology Report

Clinical Data: Left breast mass.
Nature of Specimen: Tru-cut biopsy, collected at 22/03/2023, 12:45 pm.

Gross Examination:
- Multiple grayish white tissue cores up to 1 cm, totally submitted.

Microscopic Examination:
* Histological type: Ductal carcinoma in situ (comedo, non comedo).
* No definite invasive tumor in the biopsy.
* Microcalcifications: Yes.

Diagnosis:
Left breast mass, Tru-cut biopsy:
- Ductal carcinoma in situ (comedo, non comedo).
- No definite invasive tumor in the biopsy.
- For IHC (ER).

* Comment: Extended biopsy is advisable to rule out nearby invasion.

Signature
Prof. Dr. Rabna Omar
Dr. Menna Nabil

Verified by :
DR.Menna Nabil

Name: منصورة عدلي احمد محمد MR. : 319	

Detailed Medical Report

السيد	

تحية طيبة وبعد,,

إيماء للطلب المقدم من		بشأن تقرير طبي للمريض	
بقسم:		رقم طبي:	

نتشرف بأن نوضح فيما يلي تقريرا عن حالتها

تاريخ وساعة الدخول :		تاريخ وساعة الخروج :	

وبعد الكشف والفحوصات والمتابعة وجد انها

The patient presented for follow-up imaging, with a computed tomography of the chest and abdomen performed in August 2025 reported as free of disease. A magnetic resonance imaging of the brain was also obtained and was negative for metastatic involvement. The patient was deemed free for supportive treatment at this time.

On 13 June 2023 the patient underwent left-sided breast surgery consisting of a lumpectomy with sentinel lymph node biopsy. Pathology revealed invasive ductal carcinoma with negative surgical margins. One of three sentinel lymph nodes was positive for metastasis. Subsequent staging imaging, including a CT of the chest and abdomen in August 2025 and an APUS study in April 2026, were both reported as free of disease. The current plan is continued surveillance and supportive care.

تحريرا في ,

التاريخ :	19-09-2026	الوقت :	
الطبيب المعالج :	Reham Ahmed Fathy Ibrahim		

يعتمد ,

Patient Name	: الأستاذه / منصوره عادلى احمد		
Patient No.	: 26811182700122	Request No.	: 13008862
Path No.	: 1632/23	Request Date	: 09/07/2023
Sex	: Female	Verification Date	: 19/07/2023
Age	: 54 Y	Financial Category	: اورام دار السلام - نفقة الدولة
Referreing Doc.	:	Department	: Out Patient
		IHC No.	:
		Registered by	: .marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Left breast cancer, Up-Front.
Nature of Specimen: Left simple mastectomy and SLNs.

Gross Examination:
Procedure: Left simple mastectomy specimen measured 23x21x5 cm with covering skin ellipse measured 21x14 cm.
Cut section revealed 2 masses. First mass 4x3.5x2 cm, away from deep margin by 3.5 cm and 2 cm from skin located in the upper outer quadrant. Second mass 1x1x0.5 cm away from deep margin by 2 cm, located centrally. Distance inbetween two masses is 4 cm.
Skin involvement: Grossly unremarkable nipple and areola.
Sentinal LNs: Fibrofatty tissue , entangling 3 lymph nodes up to 1.2x1 cm.
Frozen results: One nodule was suspiceous (wait for paraffin)
Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for ER, PR, Her2, ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Microscopic Examination:
Histological type: Invasive duct carcinoma.
Grade (differentiation+ nuclear pleomorphism+ mitosis):
Grade II (3+2+1)
Angioinvasion: No .
Ductal carcinoma in situ: Yes, high grade (60%).
Skin involvement: Free .
Lymph node involvement: Positive in 1/3 of SLN .
Resection Margins: Free deep margin .
IPT (Positive control showed appropriate reactivity):
ER: No nuclear reaction in tumor cells (Allred score 0/8).
PR: No nuclear reaction in tumor cells (Allred score 0/8).

Verified by :
Dr.Hebatallah Mostafa

Women's Health Outreach Program

Name: Mansoura Adly Ahmed
Date 22/3/2023

Bilateral Full field Digital Mammography

clinical summary :55 years old left breast bloody nipple discharge.

Technique:
Mediolateral oblique and CC views
High resolution ultrasound.

Detailed Findings:

Right breast:

- scattered fibroglandular parenchyma (ACR b).
- No asymmetric densities , distortion or suspicious clusters of microcalcifications.
- Ultrasound no solid or cystic lesions.
- prominent retro-areolar ducts.
- Normal skin/nipple/areolar complex.
- Non-specific axillary LNs.

Left breast:

- scattered fibroglandular parenchyma (ACR b).
- Upper outer quadrant focal asymmetry extending to the nipple with suspecious microcalcifications in segmental disturbition .No definite mass or spiculations.
- Ultrasound demonstrated corresponding area of dilated ducts extended from nipple to the UOQ more evident at 3 o'clock.
- Non-specific axillary LNs.

Impression:
Right breast (.BIRADS 2) prominent retro-areolar ducts.
Left breast (.BIRADS 4) UOQ asymmetry with suspecious microcalcifications for ultrasound guided biopsy and histopathological assessment.

Regards,
Dr/Eman Ali
Dr christina Wagih

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصحة والسكان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Patient Name: Mansourah Adli Ahmed Mohamed	Date : 08/07/2023
Exam : C.T ABDOMEN &PELVIS	Age : 54 Year(s)
Medical ID: 1424/2023	Pat. ID: 39913

NON-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN, AND PELVIS: -

Findings:

Known case of breast cancer

I Chest: -

- Left MRM with small seroma.
- Bilateral osaic attenuation of both lungs.
- Middle lobe pulmonary nodule measuring about 6 mm.
- Lingular segemnt consilidative patchy opacity is noted.
- Prominent hyperdense mediasinal lymph nodes likley post granulomatous.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.

II Abdomen & pelvis:

- The liver is of average size with homogenous CT texture. No focal lesions on non contrast basis or contour irregularities.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.
- No lytic or sclerotic osseous deposits.


جمهورية مصر العربية
بطاقة تحقيق الشخصية
متصوره
عادلى احمد محمد
٩ عبدالصمد علبى-اسامة ابو عميرة-فيصل
الهرم - الجيزه
١٩٦٨/١
٢٢٠٠١٢٧١٨١١٦٨٢
KJ2430860

٢٠٢٤/٠١
بدون عمل
الشي
مسلمة
إرملة
البطاقة سارية حتى ٢٠٣١/٠١/١٥

GUSTAVE ROUSSY
INTERNATIONAL

Patient Name: Mansourah Adli Ahmed Mohamed	Date : 13/11/2025
Exam : ultrasound BREAST (BOTH SIDE)	Age : 57 Year(s)
Medical ID: 1424/2023	Pat. ID: 39913

HIGH RESOLUTION ULTRASOUND

INDICATION:

- Left MRM

REPORT:

- Normal echotexture of the right breast.
- No right breast solid or cystic masses.
- Normal skin thickness and nipple-areola complex.
- Bilateral nonspecific axillary nodes, with preserved fatty hila and uniform cortical thickness.
- Free right operative bed with no evidence of recurrent mass lesions, no pathological axillary node and free infra and supra clavicular regions.

OPINION:

- Right MRM with free operative bed.
- Normal sonographic study of the left breast, (BIRADS 1).

Best regards,
Dr. Mahi Mokhtar

GUSTAVE ROUSSY INTERNATIONAL EGYPT

Patient Name: Mansourah Adli Ahmed Mohamed	Date : 24/08/2025
Exam : MRI BRAIN WITH CONTRAST	Age : 56 Year(s)
Medical ID: 1424/2023	Pat. ID: 39913

MRI OF THE BRAIN

TECHNIQUE:

- **Sagittal T2WI**
- **Axial T1, T2 and FLAIR WI**
- **Coronal T1WI**
- **Post contrast T1 WIs**

FINDINGS:

- Normal size and parenchymal signal intensity pattern of the cerebral hemispheres, cerebellum and brain stem.
- No intra or extra axial S.O.Ls.
- No areas of pathological enhancement.
- Normal size and configuration of the ventricular system.
- No shift of the midline structures.
- No extra-axial collections.
- Intact cranio-cervical junction.

OPINION:

- **Normal contrast enhanced MRI of the brain.**

Dr.Sara Mahmoud, MD

GUSTAVE ROUSSY
INTERNATIONAL
EGYPT

Patient Name: Mansourah Adli Ahmed Mohamed	Date : 20/09/2025
Exam : MRI HIBS	Age : 57 Year(s)
Medical ID: 1424/2023	Pat. ID: 39913

MRI EXAMINATION OF THE BOTH HIP

Findings:

- Osteo arthritic changes of hip joint with subtle joint effusion.
- Narrowing of the ischio-femoral spaces on both sides being 9 mm on right side and 11 mm on left side with abnormal intra substance high STIR SI noted within quadratus femoris muscle
- Abnormal intra substance high T2 signal noted within gluteus medius & maximus muscles at their insertion into greater trochanter
- Normal configuration and marrow signal intensity of femoral head showing normal spherical contour and smooth outline with no evidence of cortical irregularity or structural collapse.
- Subtle joint effusion
- No pelvic soft tissue masses detected
- Normal marrow signal of the femur with no evidence of marrow infiltrative lesions.
- No evidence of masses

CONCLUSION:

- **Osteo arthritic changes of hip joint**
- **Evidence of ischio femoral impingement with quadratus femoris muscle strain**
- **Greater trochanteric pain syndrome .**
- **Minimal joint effusion.**

Dr.Reham Khedr (MD)

GUSTAVE ROUSSY
INTERNATIONAL

NAME: Mansoura adly ahmed
DATE;12/4/2026

ABDOMINO-PELVIC ULTRASOUND EXAMINATION REVEALED:

Liver: -

- *Average in size (15cm at Rt MCL) bright parenchymal echo pattern with smooth surface.*
- *No hepatic focal lesions or intra hepatic biliary radicle dilatation.*
- *Portal vein is patent and not dilated.*

Gall Bladder: -

- Average distention and capacity with no stones or mud
- *. The wall is not thickened.*
- *CBD is not dilated.*

Spleen: -

Average size, homogenous.

- **Kidneys: -**
- *Both are average in size,shape and echogenicity*
- *No stones or dilated pelvicalyceal system*
- *No gross lesion*

Para-aortic and pancreatic area*: show no gross abnormalities.*

No ascites

Urinary Bladder: -

- *Not distended enough however no stones or diverticular out pouching*

Uterus and adnexal; *no abnormalities could be detected*

Opinion:

Bright liver likely fatty
Colonic gaseous distention

Best regards
Dr. NERMEEN RAMADAN FEBR. MD

Presidential Initiative for Women's Health - Pathology Lab

المبادرة الرئاسية لصحة المرأة - معمل الباثولوجي

Patient Name	: الأستاذه / منصوره عادلى احمد	Request No.	: 13002182
Patient No.	: 26811182700122	Request Date	: 23/03/2023
Path No.	: 914/2023	Verification Date	: 09/04/2023
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 54 Y	Department	: Out Patient
Referreing Doc :		Registered by	: Ali mostafa

Pathology Report

Clinical Data: Left breast mass.
Nature of Specimen: Tru-cut biopsy, collected at 22/03/2023, 12:45 pm.

Gross Examination:
- Multiple grayish white tissue cores up to 1 cm, totally submitted.

Microscopic Examination:
* Histological type: Ductal carcinoma in situ (comedo, non comedo).
* No definite invasive tumor in the biopsy.
* Microcalcifications: Yes.

Diagnosis:
Left breast mass, Tru-cut biopsy:
- Ductal carcinoma in situ (comedo, non comedo).
- No definite invasive tumor in the biopsy.
- For IHC (ER).

* Comment: Extended biopsy is advisable to rule out nearby invasion.

Signature
Prof. Dr. Lubna Omar
Dr. Menna Nabil

Verified by :
DR.Menna Nabil

قطعة رقم (4) - الدور الأول - مبنى رقم (937) - شارع الثلاثين - مجاور كلية - القاهرة - مصر
Room No. (4) - First Floor Building No. 937 - 30th Corniche - After El Qadasia - Cairo, Egypt

Women's Health Outreach Program

Name: Mansoura Adly Ahmed
Date 22/3/2023

Bilateral Full field Digital Mammography

clinical summary :55 years old ,left breast bloody nipple discharge.

Technique:
Mediolateral oblique and CC views
High resolution ultrasound

Detailed Findings:

Right breast:

- scattered fibroglandular parenchyma (ACR b).
- No asymmetric densities , distortion or suspicious clusters of microcalcifications.
- Ultrasound no solid or cystic lesions
- prominent retro-areolar ducts.
- Normal skin/nipple/areolar complex.
- Non-specific axillary LNs .

Left breast:

- scattered fibroglandular parenchyma (ACR b).
- Upper outer quadrant focal asymmetry extending to the nipple with suspecious microcalcifications in segmental disturbition .No definite mass or spiculations.
- Ultrasound demonstrated corresponding area of dilated ducts extended from nipple to the UOQ more evident at 3 o'clock.
- Non-specific axillary LNs .

Impression:
Right breast (BIRADS 2) prominent retro-areolar ducts.
Left breast (BIRADS 4) UOQ asymmetry with suspecious microcalcifications for ultrasound guided biopsy and hist pathological assessment.

Regards,
Dr/Eman Ali
Dr christina Wagih

Patient Name	: الأستاذه / منصوره عادلى احمد	Request No.	: 13008862
Patient No.	: 26811182700122	Request Date	: 09/07/2023
Path No.	: 1632/23	Verification Date	: 19/07/2023
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 54 Y	Department	: Out Patient
Referreing Doc.	:	IHC No.	:
		Registered by	: .marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Left breast cancer, Up-Front.
Nature of Specimen: Left simple mastectomy and SLNs.

Gross Examination:
Procedure: Left simple mastectomy specimen measured 23x21x5 cm with covering skin ellipse measured 21x14 cm.
Cut section revealed 2 masses. First mass 4x3.5x2 cm, away from deep margin by 3.5 cm and 2 cm from skin located in the upper outer quadrant. Second mass 1x1x0.5 cm away from deep margin by 2 cm, located centrally. Distance inbetween two masses is 4 cm.
Skin involvement: Grossly unremarkable nipple and areola.
Sentinal LNs: Fibrofatty tissue , entangling 3 lymph nodes up to 1.2x1 cm.
Frozen results: One nodule was suspiceous (wait for paraffin)
Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for ER, PR, Her2, ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Microscopic Examination:
Histological type: Invasive duct carcinoma.
Grade (differentiation+ nuclear pleomorphism+ mitosis):
Grade II (3+2+1)
Angioinvasion: No .
Ductal carcinoma in situ: Yes, high grade (60%).
Skin involvement: Free .
Lymph node involvement: Positive in 1/3 of SLN .
Resection Margins: Free deep margin .
IPT (Positive control showed appropriate reactivity):
ER: No nuclear reaction in tumor cells (Allred score 0/8).
PR: No nuclear reaction in tumor cells (Allred score 0/8).

Verified by :
Dr.Hebatallah Mostafa

Manor No. (6) - First Floor/Building No. 997- Nile Corniche - Misr El Qadeem- Cairo, Egypt

Presidential Initiative for Women's Health - Pathology Lab

المبادرة الرئاسية لصحة المرأة - معمل الباثولوجي

Patient Name	: الأستاذه / منصوره عادلى احمد	Request No.	: 13008862
Patient No.	: 26811182700122	Request Date	: 09/07/2023
Path No.	: 1632/23	Verification Date	: 19/07/2023
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 54 Y	Department	: Out Patient
Referring Doc.	:	IHC No.	:
		Registered by	: .marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Left breast cancer, Up-Front.
Nature of Specimen: Left simple mastectomy and SLNs.

Gross Examination:
Procedure: Left simple mastectomy specimen measured 23x2 x5 cm with covering skin ellipse measured 21x14 cm.
On section revealed 2 masses. First mass 4x3.5x2 cm, away from deep margin by 3.5 cm and 2 cm from skin located in the upper outer quadrant. Second mass 1x1x0.5 cm away from deep margin by 2 cm, located centrally. Distance inbetween two masses is 4 cm.
Skin involvement: Grossly unremarkable nipple and areola.
Sentinal LNs: Fibrofatty tissue , entangling 3 lymph nodes up to 1.2x1 cm
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Angioinvasion: No .
Ductal carcinoma in situ: Yes, high grade (60%).
Skin involvement: Free .
Lymph node involvement: Positive in 1/3 of SLN .
Resection Margins: Free deep margin .
IPT (Positive control showed appropriate reactivity):
ER: No nuclear reaction in tumor cells (Allred score 0/8).
PR: No nuclear reaction in tumor cells (Allred score 0/8).

Verified by :
Dr.Hebatallah Mostafa

Nasser No. 60 - First Floor-Building No. 107 - 15th Corniche - Sidi Al Qalaan Cairo, Egypt

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصـــــحة والسكـــان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Patient Name: Mansourah Adli Ahmed Mohamed	Date : 08/07/2023
Exam : C.T ABDOMEN &PELVIS	Age : 54 Year(s)
Medical ID: 1424/2023	Pat. ID: 39913

Opinion:

Middle lobe nodule for clinical correlation and close follow up.
No definite hepatic metastatic deposits on non-contrast bases.

Signature Prof. Menna Tallah Hanafy
10/07/2023 05:57 م

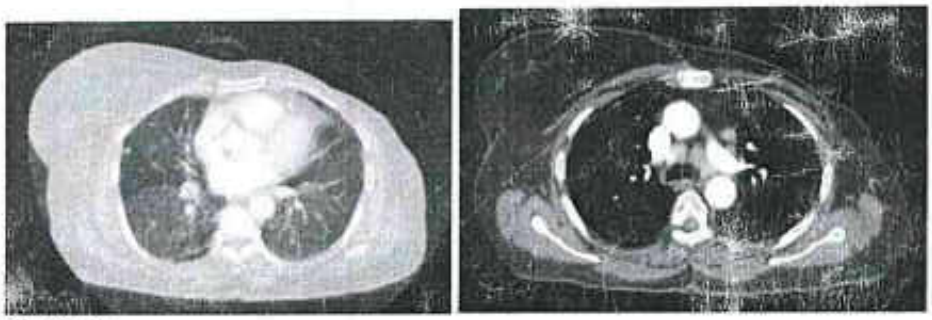
GUSTAVE ROUSSY
INTERNATIONAL
EGYPT

Patient Name: Mansourah Adli Ahmed Mohamed	Date : 24/08/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 56 Year(s)
Medical ID: 1424/2023	Pat. ID: 39913

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST: -

Clinical History:

- ***Known case of left breast cancer, compared to the previous study report dated 1-3-2025, the current study revealed:***



Findings:

- Clear left MRM operative bed with no sizable soft tissue masses to account for residual/recurrent neoplasia.
- Few subcentimetric left axillary lymph nodes are noted. the largest seen m measuring 11x5.5 mm.
- Few right axillary lymph nodes are noted, the largest measuring 16x12 mm.
- Still noted mildly hyperdense mediasinal and bilateral hilar lymph nodes, the largest still noted at the left paratracheal group currently measuring 20x18 mm (compared to 18x15 mm).
- Still noted mosaic attenuation of both lungs.
- Still noted middle lobe pulmonary nodule measuring about 6 mm.
- Still noted lingular subpleural reticular opacities likely post irradiation sequel.
- Patent tracheo-bronchial tree.
- Normal CT appearance of the mediastinal vessels.
- Average size of the heart.
- No evidence of pleural or pericardial effusion.

OPINION:

- ***Clear left MRM operative bed with no sizable soft tissue masses to account for residual/recurrent neoplasia.***
- ***Rather stationary course of the disease process.***

Marina Essam, MSc
Dr.Noura Nabil,MD

v

Signature Noura Nabil
28/08/2025 1:19 AM

GUSTAVE ROUSSY
INTERNATIONAL
EGYPT

Patient Name: Mansourah Adli Ahmed Mohamed	Date : 24/08/2025
Exam : MRI BRAIN WITH CONTRAST	Age : 56 Year(s)
Medical ID: 1424/2023	Pat. ID: 39913

MRI OF THE BRAIN

TECHNIQUE:

- **Sagittal T2WI**
- **Axial T1, T2 and FLAIR WI**
- **Coronal T1WI**
- **Post contrast T1 WIs**

FINDINGS:

- Normal size and parenchymal signal intensity pattern of the cerebral hemispheres, cerebellum and brain stem.
- No intra or extra axial S.O.Ls.
- No areas of pathological enhancement.
- Normal size and configuration of the ventricular system.
- No shift of the midline structures.
- No extra-axial collections.
- Intact cranio-cervical junction.

OPINION:

- **Normal contrast enhanced MRI of the brain.**

Dr.Sara Mahmoud, MD

Presidential Initiative for Women's Health - Pathology Lab

المبادرة الرئاسية لصحة المرأة - معمل الباثولوجي

Patient Name	: الأستاذه / منصوره عادلى احمد	Request No.	: 13002182
Patient No.	: 26811182700122	Request Date	: 23/03/2023
Path No.	: 914/2023	Verification Date	: 09/04/2023
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 54 Y	Department	: Out Patient
Referring Doc :		Registered by	: Ali mostafa

Pathology Report

Clinical Data: Left breast mass.
Nature of Specimen: Tru-cut biopsy, collected at 22/03/2023, 12:45 pm.

Gross Examination:
- Multiple grayish white tissue cores up to 1 cm, totally submitted.

Microscopic Examination:
* Histological type: Ductal carcinoma in situ (comedo, non comedo).
* No definite invasive tumor in the biopsy.
* Microcalcifications: Yes.

Diagnosis:
Left breast mass, Tru-cut biopsy:
- Ductal carcinoma in situ (comedo, non comedo).
- No definite invasive tumor in the biopsy.
- For IHC (ER).

* Comment: Extended biopsy is advisable to rule out nearby invasion.

Signature
Prof. Dr. Lubna Omar
Dr. Menna Nabil

Verified by :
DR.Menna Nabil

Menna No. 44 - First Floor Building No. 937 - 9th Corniche - Nile Cornich - Nile St Qasr an - Cairo, Egypt

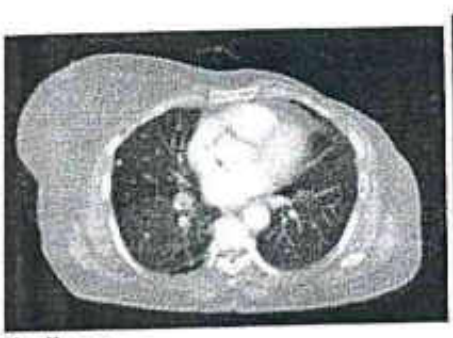
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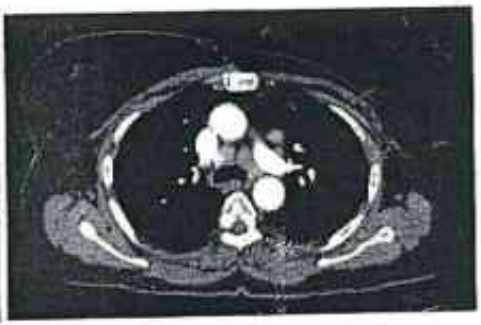
Patient Name: Mansourah Adli Ahmed Mohamed	Date : 24/08/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 56 Year(s)
Medical ID: 1424/2023	Pat. ID: 39913

OST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST: -

linical History:

- ***Known case of left breast cancer, compared to the previous study report dated 1-3-2025, the current study revealed:***





Findings:

- Clear left MRM operative bed with no sizable soft tissue masses to account for residual/recurrent neoplasia.
- Few subcentimetric left axillary lymph nodes are noted, the largest seen m measuring 11x5.5 mm.
- Few right axillary lymph nodes are noted, the largest measuring 16x12 mm.
- Still noted mildly hyperdense mediasinal and bilateral hilar lymph nodes, the largest still noted at the left paratracheal group currently measuring 20x18 mm (compared to 18x15 mm).
- Still noted mosaic attenuation of both lungs.
- Still noted middle lobe pulmonary nodule measuring about 6 mm.
- Still noted lingular subpleural reticular opacities likely post irradiation sequel.
- Patent tracheo-bronchial tree.
- Normal CT appearance of the mediastinal vessels.
- Average size of the heart.
- No evidence of pleural or pericardial effusion.

OPINION:

- ***Clear left MRM operative bed with no sizable soft tissue masses to account for residual/recurrent neoplasia.***
- ***Rather stationary course of the disease process.***

Marina Essam, MSc
Dr.Noura Nabil,MD

Signature Noura Nabil
28/08/2025 1:19 AM

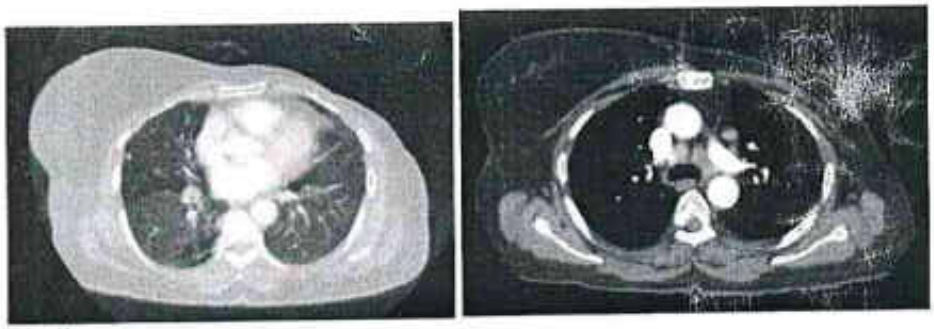
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INTERNATIONAL
EGYPT

Patient Name: Mansourah Adli Ahmed Mohamed	Date : 24/08/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 56 Year(s)
Medical ID: 1424/2023	Pat. ID: 39913

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST: -

Clinical History:

- ***Known case of left breast cancer, compared to the previous study report dated 1-3-2025, the current study revealed:***



Findings:

- Clear left MRM operative bed with no sizable soft tissue masses to account for residual/recurrent neoplasia.
- Few subcentimetric left axillary lymph nodes are noted, the largest seen m measuring 11x5.5 mm.
- Few right axillary lymph nodes are noted, the largest measuring 16x12 mm.
- Still noted mildly hyperdense mediasinal and bilateral hilar lymph nodes, the largest still noted at the left paratracheal group currently measuring 20x18 mm (compared to 18x15 mm).
- Still noted mosaic attenuation of both lungs.
- Still noted middle lobe pulmonary nodule measuring about 6 mm.
- Still noted lingular subpleural reticular opacities likely post irradiation sequel.
- Patent tracheo-bronchial tree.
- Normal CT appearance of the mediastinal vessels.
- Average size of the heart.
- No evidence of pleural or pericardial effusion.

OPINION:

- ***Clear left MRM operative bed with no sizable soft tissue masses to account for residual/recurrent neoplasia.***
- ***Rather stationary course of the disease process.***

Marina Essam, MSc
Dr.Noura Nabil,MD

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Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

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أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Patient Name: Mansourah Adli Ahmed Mohamed	Date : 08/07/2023
Exam : C.T ABDOMEN &PELVIS	Age : 54 Year(s)
Medical ID: 1424/2023	Pat. ID: 39913

NON-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN, AND PELVIS: -

Findings:

Known case of breast cancer

I Chest: -

- Left MRM with small seroma.
- Bilateral osaic attenuation of both lungs.
- Middle lobe pulmonary nodule measuring about 6 mm.
- Lingular segemnt consilidative patchy opacity is noted.
- Prominent hyperdense mediasinal lymph nodes likley post granulomatous.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.

II Abdomen & pelvis:

- The liver is of average size with homogenous CT texture. No focal lesions on non contrast basis or contour irregularities.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.
- No lytic or sclerotic osseous deposits.

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ادارة الاشعة

Patient Name: Mansourah Adli Ahmed Mohamed	Date : 04/06/2024
Exam : ultrasound BREAST (BOTH SIDE)	Age : 55 Year(s)
Medical ID: 1424/2023	Pat. ID: 39913

DIGITAL SOFT TISSUE MAMMOGRAPHY AND COMPLEMENTARY ULTRASOUND

Indication:

Patient is seen for follow-up after left MRM

Report:

- Left MRM:
  - Operative bed shows no mass or collection
  - No regional lymphadenopathy.
- Right breast:
  - Predominant Fatty parenchyma (ACR A)
  - No suspicious micro calcifications or architectural distortion detected.
  - Complementary ultrasound no cystic or solid mass lesions.
  - Normal skin thickness, and nipple-areola complex with average caliber of the retro-areolar central ducts.
  - Nonspecific axillary LN.

Opinion:

- Post left MRM status showing no evidence of recurrence (BIRADS 2)
- Benign findings of right breast (BIRADS 1)

Best regards,
Dr. Eman Abdel Razik, MSc, FEBR
Dr. Amira H Radwan MD,
Breast Imaging consultant,
NCI, Cairo University
04/06/2024 2:12 PM